

16.

CASE #	CASE NAME	HEARING NAME
CVRI2405306	NEGRETE VS RIVERSIDE HEALTHCARE SYSTEM, L.P., D/B/A RIVERSIDE COMMUNITY HOSPITAL	DEMURRER TO FOURTH AMENDED COMPLAINT

Tentative Ruling: The Court OVERRULES Defendant's evidentiary objections in full. Plaintiffs do not offer these exhibits in opposition to this demurrer, but rather to support their request for leave to amend the underlying complaint. (Opp., pp. 12-14; Ikuta Decl., ¶ 5-10.)

The Court SUSTAINS the demurrer to the first cause of action. The Court GRANTS Plaintiffs leave to amend the first cause of action within 30 days to reflect the new facts discovered at the deposition of Julie Kleinhampf, R.N, taken June 4, 2026.

Again, Plaintiffs fail to allege any facts to establish a caretaking or custodial relationship existed between Decedent and Defendant (i.e., that Defendant had the responsibility for meeting Decedent's basic needs) to support a claim under the Elder Abuse Act. Rather, Plaintiffs merely conclude that Defendant was "a care custodian with a robust caretaking a[nd] custodial relationship, separate and apart from the mere provision of medical care" and "had a duty to closely monitor his breathing, as he would be unable to provide for his own basic needs and safety otherwise." (4AC, ¶ 15.) Plaintiffs alleged that Defendant's "relationship with the Decedent transformed into a custodial relationship upon such findings by [Defendant] that the Decedent was unable to care for his own basic needs, including safety, and required monitoring and assistance with mobility." (4AC, ¶ 18.)

However, in *Oroville Hospital v. Superior Court* (2022) 74 Cal.App.5th 382, 405, the court found that the provision of medical treatment requiring "competent professional medical attention," such as wound care, is not considered "a basic need an able-bodied and fully competent adult would be capable of managing without assistance, such as eating, taking medicine, or using the restroom," and was therefore insufficient to "give rise to the substantial caretaking or custodial relationship required to establish neglect under the Elder Abuse Act." Similarly, here, Plaintiffs' factual allegations of the specific medical services provided by Defendant, including electronic vital sign and oxygen monitoring, are not akin to services caring for "basic needs" such as eating, drinking, and toileting, which are required to establish a caretaking or custodial relationship under *Oroville* and *Kruthanooch*. Although Plaintiffs allege that Decedent required assistance for mobility and to eat (4AC, ¶ 16), Plaintiffs again fail to allege that Defendant provided any such assistance. Further, as noted above, brief assistance with one or more basic needs is insufficient to establish the robust caretaking relationship required under the Elder Abuse Act. (*Kruthanooch*, 83 Cal.App.5th at 1132.) Accordingly, as before, Plaintiffs' facts do not demonstrate that Defendant undertook a caretaking or custodial role necessary to establish a claim for neglect under the Elder Abuse Act.

1. Sham Pleading

Plaintiffs then allege that a caretaking or custodial relationship was established because Decedent on September 9, 2023, Plaintiff “was documented as continuing to be confused and agitated, significant enough to require sedation medication and family presence at his bedside,” and “[i]n his altered mental state with underlying respiratory issues, he was utterly reliant on Defendants, as his care custodians, for fundamental support in the provision of basic needs, the securing of his safety and protecting him from foreseeable harm.” (4AC, ¶ 16.)

But Plaintiffs previously alleged in the FAC that on the morning of September 9, 2023, “Decedent was seen in the morning, awake and able to follow simple commands, and he denied any complaints... The bedside nurse reported that Decedent had become agitated overnight and was given Atarax to calm him down. The family was called to be with him, and by the morning, he appeared calmer and more cooperative... He was improving.” (FAC, ¶ 21.) The Court sustained Defendant’s demurrer to the FAC based on these allegations. Similarly, the TAC alleges that “On September 9, the morning after admission, the chart does reflect that Decedent was slightly improved.” (TAC, ¶ 21.) Notably, the 4AC omits these facts and instead alleges, “On September 9, Decedent remained in an altered mental state, experiencing hallucinations, and unable to provide for his own basic safety. The only reliable way to ensure Decedent’s safety was continuous monitoring, as he could not self-report any health deterioration in his vulnerable, delirious state.” (4AC, ¶ 19.) The factual allegations in the 4AC directly contradict Plaintiffs’ prior allegations that the Decedent was awake, cooperative, improving, able to follow simple commands, and denied having any complaints. Further, as Defendant points out, the 4AC omits numerous factual allegations characterizing Defendant’s services, such electronic monitoring of Decedent’s oxygen saturation and pulse, as oxygen delivery, and breathing treatments, as “essential medical care.” (See TAC, ¶ 11-12, 14, 20, 23.)

Plaintiffs’ omission of previously pleaded facts that were harmful to their prior complaints raises a sham pleading issue. “[W]here an amended complaint attempts to avoid defects set forth in a prior complaint by ignoring them[, t]he court may examine the prior complaint to ascertain whether the amended complaint is merely a sham.” (*Kenworthy v. Brown* (1967) 248 Cal.App.2d 298, 302.) “A pleader may not attempt to breathe life into a complaint by omitting relevant facts which made his previous complaint defective. Moreover, any inconsistencies with prior pleadings must be explained; if the pleader fails to do so, the court may disregard the inconsistent allegations. Accordingly, a court is not bound to accept as true allegations contrary to factual allegations in former pleading in the same case.” (*Vallejo Development Co. v. Beck Development Co.* (1994) 24 Cal.App.4th 929, 946 [internal punctuation and citations omitted].)

Plaintiffs attempt to explain the inconsistencies in the prior allegations by stating that they merely “streamlined those allegations to present a more organized pleading for both Defendant the Court.” (Opp., p. 2.) However, Plaintiffs’ omission of previously pleaded facts that were harmful to their claim releases the Court of its obligation to

accept that the 4AC's allegations of Decedent's condition on the morning of September 9, 2023, are true in evaluating this demurrer. Further, the Court may disregard the 4AC's inconsistent allegations to the extent that it attempts to characterize Defendant's "duty to closely monitor his breathing" as "separate and apart from the mere provision of medical care." (4AC, ¶ 15.)

Even if the omitted facts do not render the 4AC a sham pleading, Plaintiffs' new allegations still fail to establish that Decedent's allegedly vulnerable state gave rise to a caretaking or custodial relationship. Even assuming the new allegations of Decedent's condition on the morning of September 9, 2023, are true, the fact that Decedent "was dependent and vulnerable as a general matter does not mean that anyone who entered into [his] orbit and provided [him] with a service would have entered into a caretaking or custodial relationship with [him]." (*Oroville*, 74 Cal.App.5th at 406.) In this case, Plaintiffs allege that Decedent's "vulnerable, delirious state" required him to remain connected to the continuous monitoring systems, but Defendant's staff disconnected Decedent from the monitoring systems and failed to adequately monitor him despite the disconnection. (4AC, ¶ 20.) However, the facts are insufficient to show that Decedent's need for medical treatment (i.e., supplemental oxygen and continuous electronic monitoring) supports a finding of a caretaking or custodial relationship.

Accordingly, Plaintiffs still fail to establish the existence of a caretaking or custodial relationship between Defendant and Decedent.

A. Custodial Neglect vs. Professional Negligence

The facts of the instant case mirror those in *Faiaipau v. THC-Orange County, LLC* (2025) 117 Cal.App.5th 292 rather than *Holland v. Silverscreen Healthcare, Inc.* (2025) 18 Cal.5th 364. Specifically, although the 4AC contains some allegations that Decedent required assistance with toileting and eating, the primary action that Plaintiffs allege caused Decedent's death is the disconnection of the electronic monitoring system that monitored Decedent's respiratory status. As in *Faiaipau*, the provision of an electronic monitoring system is, "on its face, medical care, not a custodial act incidental to medical care," and "failing to monitor and reconnect a[n electronic monitoring system] is professional negligence, not custodial neglect." (*Faiaipau*, supra, 117 Cal.App.5th at 304-305.) Further, similar to *Faiaipau*, Plaintiffs' contention that Defendant's omission in failing to have staff observe and check on Decedent (4AC, ¶ 20) is not a failure to provide medical care, but rather a negligent undertaking to provide electronic monitoring because Defendant failed to pay attention to the monitoring it was already providing and ensure it continued to function properly and give Decedent the assistance that he needed. (*Faiaipau*, 117 Cal.App.5th at 305.)

Plaintiffs contend that Defendant's disconnection of the electronic monitoring system constitutes custodial neglect, specifically sporadic care, and points to the *Faiaipau* court's discussion of *Sababin v. Superior Court* (2006) 144 Cal.App.4th 81, 83-84, in which Plaintiffs assert that the *Faiaipau* court "would have upheld the trial court's determination that the complaint sounded in elder abuse if [defendant] provided [decedent] on a ventilator on some days but not others" under *Sababin*. (Opp., p. 8.) However, the *Faiaipau* court stated no such thing. Rather, the court merely acknowledged the plaintiffs' *Sababin* argument by stating, "We will assume for the sake

of argument that this reasoning is correct, but it has no application here.” (*Faiaipau*, 117 Cal.App.5th at 306.) Specifically, the *Faiaipau* court noted that the disconnection of the ventilator did not constitute sporadic care because plaintiffs did not allege that the decedent was provided a ventilator on some days but not others; rather, the disconnection was a failure to take due care in checking to ensure that the electronic monitoring system was operating correctly, which is negligence. (*Faiaipau* 117 Cal.App.5th at 305-306.) Similarly, here, although the 4AC conclusively alleges that Defendant provided sporadic electronic monitoring, the factual allegations do not state that Defendant provided the monitoring on some days but not others, but rather only allege that Defendant’s staff disconnected the monitoring system once for an extended period of time. (4AC, ¶ 20, 24, 26.) Further, *Sababin* is not controlling because it involved a rehabilitation center rather than an acute care hospital, and as such, a custodial relationship was established there that is not present here. (*Sababin* 144 Cal.App.4th at 84.)

Given the above, Plaintiffs fail to allege sufficient facts to show that Defendant’s alleged disconnection of Decedent’s electronic monitoring system constituted custodial neglect rather than professional negligence.

B. Negligence Per Se

In the context of the first cause of action for violation of the Elder Abuse Act, “neglect” is specifically defined as “[t]he negligent failure or any person having the *care or custody* of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.” (Welf. & Inst. Code §15610.57(a)(1) [emphasis added].) As discussed in detail above, Plaintiffs have failed to make a threshold showing that a caretaking or custodial relationship existed between Defendant and Decedent to establish a cause of action for neglect in violation of the Elder Abuse Act. To the extent that Plaintiffs allege that Defendants’ statutory violations support a claim for general negligence via the negligence per se doctrine, this argument applies to Plaintiffs’ third cause of action for negligence, which is not at issue in this demurrer.

Negligent Infliction of Emotional Distress

The Court STRIKES the fourth cause of action for negligent infliction of emotional distress. (C.C.P. § 436(b).) Plaintiffs failed to seek leave of Court to amend the 3AC to add their fourth cause of action for negligent infliction of emotional distress. Although Plaintiffs concurrently move to file a Fifth Amended Complaint (“5AC”) along with its opposition to the underlying demurrer, Plaintiffs only address the proposed amendments as to their first cause of action for elder abuse. Plaintiffs fail to specify the effect of the amendment of adding the fourth cause of action, why the amendment is necessary and proper, when the facts giving rise to the amended allegations were discovered, and the reasons why the request for amendment were not made earlier, as required to support a formal motion for leave to amend a pleading under California Rules of Court, Rule 3.1324(b). Accordingly, the Court exercises its discretion to strike Plaintiffs’ fourth cause of action for negligent infliction of emotional distress under C.C.P. § 436(b) because it was not filed in conformity with Rule 3.1324 or a court order.